

Emergency Department and Imaging Records

Facility: Alder Creek Regional Medical Center **Patient:** Mara Ellison **Service date:** February 3, 2025 **Civil exhibit lodged:** November 21, 2025

This synthetic compilation contains the emergency-department triage note, clinician examination, radiology reports, treatment summary, and discharge instructions associated with Ellison's arrival after a welfare-check seizure. No named clinician is a real person. Clinical histories are expressly attributed to their sources and do not decide the disputed mechanics of police contact.

Ellison arrived by ambulance awake and hemodynamically stable. The handoff described one reported drive-stun cycle without probe deployment, a controlled descent or takedown, a left-wrist abrasion, two superficial contact marks, and left-shoulder pain. Paramedic Mateo Cole reported intact hand circulation and sensation throughout transport. No weapon or criminal charge accompanied the transfer.

Triage recorded pain of six on a ten-point scale with shoulder movement and three at rest. Ellison identified the shoulder and wrist as the principal areas of discomfort. She denied loss of consciousness, severe headache, neck pain, chest pressure, shortness of breath, focal weakness, or numbness. Her immediate goal was evaluation, dressing of the contact sites, and help regaining a sense of calm.

Treatment history and competing event descriptions

Ellison told the emergency clinician that Officer Nolan Rusk grasped her left wrist, she pulled away because of pain, and he applied the device while taking her down. She denied hitting him or reaching for his equipment. The chart labels that passage “history from patient.” It is relevant to symptom location but not an independently witnessed fact.

The ambulance handoff also conveyed Rusk’s statement that Ellison twisted, struck his forearm, and reached toward his duty belt. Officer Priya Vega’s limited observation was summarized: Vega assisted after moving closer but did not reliably see Ellison’s hands during the critical interval. The clinician did not choose between those accounts and did not review camera footage during treatment.

Ellison reported a history of an episodic mental-health condition managed through outpatient care. She said she had missed medication and became overwhelmed after a difficult day. She denied intentional self-harm, ingestion, or a plan to harm another person. A crisis clinician was requested for a separate assessment. The emergency note avoids treating diagnosis as a proxy for violence.

Past medical history disclosed no prior fracture or surgery of the left wrist or shoulder. Ellison reported no anticoagulant use and no relevant medication allergy. The clinician explained that imaging could assess bone and alignment but would not reconstruct hand movements hidden during police contact.

Physical examination

Ellison was alert, conversant, and tearful but increasingly calm when questions were asked one at a time. Head examination disclosed no scalp tenderness, facial injury, or sign of acute head trauma. Neck movement was comfortable. Cardiopulmonary examination was unremarkable for the purposes of this encounter, and the rhythm tracing showed no acute abnormality.

The left wrist had a superficial linear abrasion and mild circumferential tenderness without a complete ligature mark. There was no gross deformity, open deep wound, or focal snuffbox tenderness. Grip effort was limited partly by discomfort, but finger movement, sensation, radial pulse, and capillary refill were intact. The right upper extremity was nontender.

Two small superficial erythematous areas were present on the left lateral torso. Their spacing and appearance were consistent with reported close-contact device exposure. There was no charred tissue, deep burn, blistering requiring debridement, or spreading redness at examination. The surrounding chest wall was stable, and breathing did not increase local pain.

The left shoulder was tender over soft tissue with pain during active abduction and flexion. Passive motion was better tolerated. There was no visible step-off or evidence of dislocation. Distal nerve and vascular findings remained intact. The clinician's working impression was shoulder strain pending imaging, superficial wrist abrasion, and superficial contact burns.

Left-wrist radiology report

Study: Three-view left wrist radiographs **Indication:** Pain and abrasion after restraint; evaluate for fracture **Comparison:** None available

The distal radius and ulna maintain normal cortical continuity. Carpal alignment is preserved. No acute fracture line, dislocation, or destructive osseous lesion is identified. Joint spaces are maintained for age. Mild soft-tissue prominence is present near the dorsal-radial wrist without radiopaque foreign material. The images do not show a probe or embedded fragment.

Impression: No acute osseous abnormality of the left wrist. Mild nonspecific soft-tissue swelling.

The radiology report addresses bone and visible soft-tissue contours. It does not determine how the abrasion occurred, quantify grip pressure, or establish whether pulling was voluntary, reflexive, threatening, or nonthreatening. A negative fracture study is consistent with the record's injury boundary and does not state that Ellison experienced no pain.

The emergency clinician reviewed the result with Ellison. Because focal circulation and sensation remained normal and no fracture appeared, rigid immobilization was not required. A removable support was offered for comfort during ordinary activity. Ellison was told to seek reassessment for increasing swelling, numbness, color change, or persistent focal pain. These precautions reflect clinical uncertainty rather than expectation of a severe injury.

Left-shoulder radiology report

Study: Three-view left shoulder radiographs **Indication:** Pain and limited active movement after descent during restraint **Comparison:** None available

The humeral head is located normally relative to the glenoid. No acute fracture, dislocation, or separation is identified. The acromioclavicular alignment is preserved. Visualized ribs show no displaced injury. No radiopaque foreign object appears in the imaged region.

Impression: No acute bony abnormality. Clinical findings may reflect soft-tissue strain.

After review, the clinician retained the diagnosis of left-shoulder strain. The chart does not describe a torn tendon, neurological deficit, or permanent restriction. Ellison could move the arm through a functional range with discomfort and received instructions for gradual use as tolerated. A short period of rest, cold application, and outpatient review was recommended.

Radiographs cannot show the exact direction of an officer's force or the subject's hand placement during a visually blocked event. They also cannot decide whether an equipment reach occurred. The medical conclusion is limited: no fracture or dislocation was detected, and the examination supported a soft-tissue injury. That limitation preserves rather than erases the documented pain and temporary functional complaint.

Treatment, crisis assessment, and disposition

The wrist abrasion and two contact areas were gently cleansed and redressed. No surgical treatment, probe extraction, or burn debridement was indicated. Ellison accepted non-opioid analgesia after the imaging discussion. The shoulder was supported for comfort, with instruction to avoid prolonged immobilization unless a follow-up clinician advised otherwise.

A crisis clinician met with Ellison in a quiet room. Ellison denied a current plan or intent to harm herself or another person. She agreed to a short stabilization and follow-up plan beginning the next day, including medication reconciliation with her established provider and contact with Iris if symptoms worsened. The crisis assessment did not characterize the police force as reasonable or unreasonable.

Ellison was discharged from emergency care with Iris after clinical observation and safety planning. She received written return precautions for chest symptoms, fainting, increasing burn changes, weakness, numbness, severe swelling, or worsening shoulder function. Follow-up was scheduled or recommended within several days.

Final emergency diagnoses were left-wrist abrasion, two superficial contact burns, left-shoulder strain, acute pain after conducted-energy exposure, and mental-health crisis addressed through outpatient stabilization. No fracture was diagnosed. No criminal booking or law-enforcement hold appears in the disposition. The record distinguishes medical discharge from any conclusion about the legal propriety of the preceding seizure force.

Records certification and interpretive boundary

The fictional health-information custodian certifies that this compilation accurately represents the emergency and imaging materials created for the February 3 encounter. The sequence reflects intake, examination, imaging, treatment, crisis review, and discharge. It omits routine billing and administrative fields that add no substantive fact to the exercise.

Statements about the police encounter are attributed to Ellison, the ambulance handoff, or an officer report. No hospital employee witnessed the obscured seconds. The chart therefore cannot corroborate a forearm strike or duty-belt reach merely by repeating Rusk's account, and it cannot prove their absence merely by recording Ellison's denial. Injury pattern alone does not identify the exact preceding movements.

The clinical findings are fixed: superficial wrist abrasion, two superficial contact burns, shoulder strain, pain, and no fracture. Later sources may discuss duration or causation but may not convert this exhibit into evidence of a broken bone. The absence of deep injury does not supply a constitutional conclusion.

This exhibit was generated on the simulated civil docket on November 21, 2025. Alder Creek Regional Medical Center is invented, as are its custodian and treatment identifiers. The certification authenticates only the fictional record and is not medical advice, a real health record, or an opinion on qualified immunity.